
Do Not Go Gentle by Kathleen Stock review – the case against euthanasia

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Do Not Go Gentle by Kathleen Stock review – the case against euthanasia

In this admirably clear and cogent book, the philosopher Kathleen Stock sets out the case against state-sanctioned assisted dying. Her immediate objection is to the end of life bill currently before the House of Lords, but her opposition extends to the principle in general. This is a polemic, but a polite one. Stock says she hopes that by the end of it we will share her objection to the "institutionalisation of death".

It is not a popular place to start. Polls over the past few years consistently show that around three-quarters of Britons are [in favour of assisted dying](#) for terminally ill people. But Stock has never been afraid of swimming upstream. In 2021, she [resigned from the University of Sussex](#) following protests by some staff and students over her views, set out in the book [Material Girls](#), that sex is binary and immutable and that this, rather than gender identity, should be the basis of laws to protect women.

In *Do Not Go Gentle* – the phrase is taken from Dylan Thomas's 1951 poem urging his ailing father not to accept death without a fight – Stock insists that she is not an unfeeling ascetic who thinks that bodily suffering is somehow good for you. Rather, her objection lies in the fact that, once we set up rules and protocols for managing assisted death, there will almost immediately be special pleadings and lobbying for extensions.

She has plenty of evidence to back this up. Canada has had Medical Assistance in Dying (Maid) since June 2016. Initial eligibility was confined to individuals with a "reasonably foreseeable" natural death. Since then it has been expanded to cover those who have serious and incurable but not necessarily terminal diagnoses. Legislation has also been passed – although implementation is delayed until 2027 – to allow Maid for individuals whose sole underlying condition is a mental illness. In the Benelux countries, it is already legal for a doctor to assist the death of a person who suffers unbearably from a psychological illness but has no underlying physical deficit. Even more chillingly, laws that started off by insisting that only adults could make the decision to die have ended up allowing euthanasia for extremely ill babies and children.

Stock, in her measured and reasonable way, does not seek to shock by lingering on these extreme cautionary tales. Instead, she concentrates on the more typical – and imaginable – case of a person with a terminal physical diagnosis who is seeking an assisted suicide. On the face of it this might seem like an obvious good. But Stock argues that if the patient had automatic access to expert palliative care and pain relief then they might not feel the need to seek an artificial death. Hospice treatment in Britain is patchy, expensive and relies on an insecure mix of charitable donations and NHS funding. How can we be certain that assisted dying is not being mooted as a way of relieving the state of an extra financial burden?

You could extend this analysis almost infinitely. Evidence from Canada shows that disabled people who are not terminally ill are now seeking death because the services required for sustaining a decent life at home are not forthcoming. Then there is the scenario of family members putting pressure on elderly relatives to exit early to avoid care home fees or to hasten legacies. It sounds like a scare story until Stock reminds us of the numerous instances of mortgage, pension and benefit fraud that come before the courts each year.

People are not always good or kind, and she urges us not to endorse a system based on muddled

thinking that puts vulnerable people – a category that will include most of us, eventually – at the mercy of coercion.

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